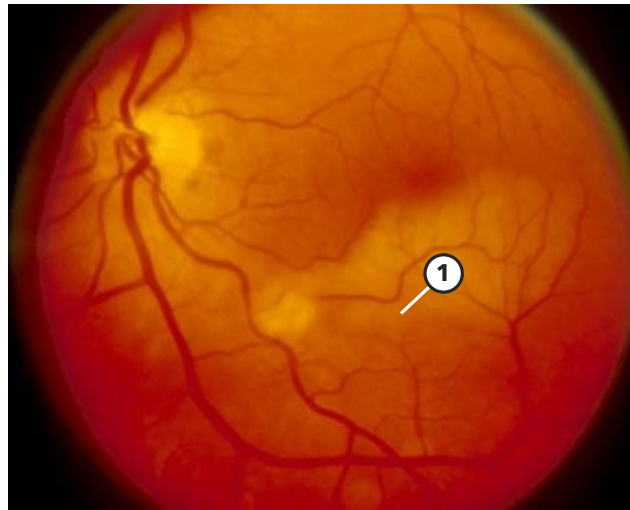


Retinal arterial territories and ocular hypoperfusion

Clinical M3-M5 | Focused Type B notes

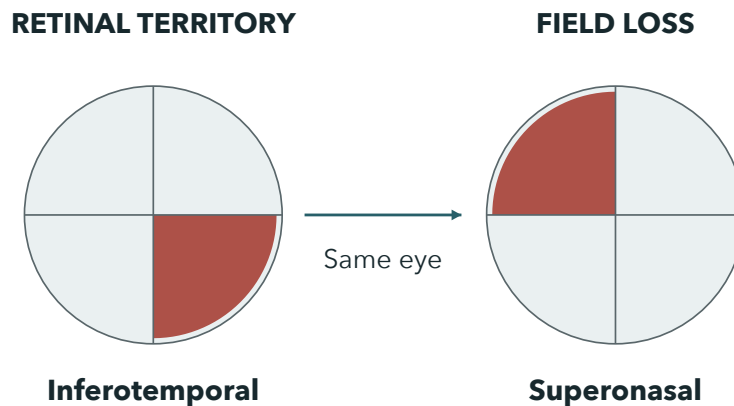
A branch lesion can spare chart acuity

BRAO: focal retinal whitening



1: whitening in a branch territory. The foveal centre can be spared, so a major field defect may coexist with useful central acuity. Supplied M3 Anki, media 232.

Retina and field are inverted



Anatomical quadrant mapping; not a photographed eye

Original anatomical mapping. Inferotemporal retinal injury maps to a superonasal field defect in the same eye.

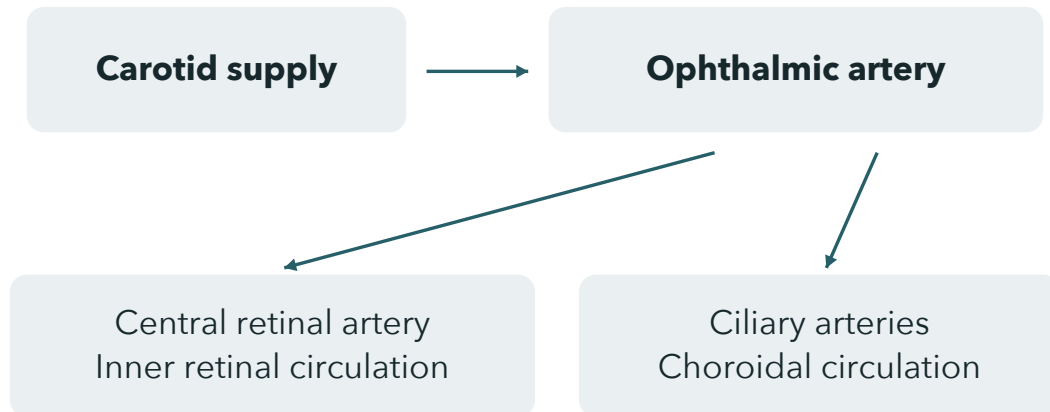
Complete the acute pathway

Arterial retinal injury still requires urgent systemic vascular assessment. Record onset, acuity, pupils and field loss; the field pattern helps localise damage but does not select a benign disposition.

References AAO: Retinal Artery Occlusions PPP, 2025

Chronic ocular ischaemia is a carotid clue

Reduced carotid supply affects retinal and choroidal perfusion



Severe inflow reduction can affect BOTH beds

Delayed choroidal filling supports ocular ischaemia

Original perfusion route. Collateral flow modifies the relationship between carotid disease and ocular findings.

OIS versus CRVO

Clue	OIS	CRVO
Veins	Dilated, but less tortuous	Markedly tortuous
Haemorrhages	Often mid-peripheral dot-blot	Diffuse venous pattern
Choroidal filling	Delayed/patchy	Usually normal

Investigate the cause

Chronic blur, ocular ache, mid-peripheral haemorrhages and delayed choroidal filling suggest ocular ischaemic syndrome. Arrange ophthalmic and carotid/vascular assessment; do not attribute every unilateral haemorrhagic retina to diabetes or CRVO.